

Understanding TRICARE For Life, Medicare Advantage & VA Reimbursement

An Educational Guide for Veterans & Military Retirees

Many veterans are covered by more than one health program at the same time — Medicare, TRICARE For Life, and VA health care. Each one works differently, and they don't always talk to each other. This guide explains, in plain language, how these programs fit together and what to do if you need to be reimbursed for out-of-pocket costs.

1. The Three Programs, in Plain Terms

- Medicare — the federal health insurance program for people 65+ (or with certain disabilities). “Original Medicare” is Parts A (hospital) and B (medical). A “Medicare Advantage” plan (Part C) is a private plan that replaces Original Medicare as your primary coverage.
- TRICARE For Life (TFL) — a benefit for TRICARE-eligible military retirees, dependents, and survivors who have Medicare Parts A and B. It acts as “wraparound” coverage, generally paying after Medicare on Medicare-covered services, with no TFL premium beyond the standard Part B premium.
- VA Health Care — direct medical care delivered through VA facilities (and, when authorized, community providers). It is a health care delivery system, not an insurance plan, and it does not bill or coordinate with Medicare or TRICARE.

2. How TRICARE For Life Works With Original Medicare

This is the pairing TFL was built around, and it's the simplest of the combinations:

- Medicare pays first on Medicare-covered services; TFL automatically pays second, generally covering the remaining copayment, coinsurance, or deductible.
- Claims cross over from Medicare to TFL automatically — in most cases, you don't file any paperwork.
- TFL includes a pharmacy benefit that counts as creditable prescription drug coverage, so you generally don't need to also carry a Part D plan.
- TFL also covers certain care overseas, which Original Medicare generally does not.

3. If You're Considering a Medicare Advantage Plan Instead

You can still keep TFL if you enroll in a Medicare Advantage plan — enrolling in one does not cause you to lose TRICARE. But the coordination changes in a few important ways:

- Medicare Advantage claims do not automatically cross over to TFL the way Original Medicare claims do.
- To be reimbursed for your share of a TRICARE-covered service, you may need to file a paper claim with TFL yourself (see Section 4).
- Medicare Advantage plans use provider networks; TFL and Original Medicare generally do not restrict you to a network. Confirm your usual providers, including any overseas or out-of-network providers you rely on, are covered before switching.

Extra benefits some Medicare Advantage plans offer

Part of why veterans consider Medicare Advantage is the extra benefits many plans include beyond what Original Medicare and TFL provide. These vary a lot by plan and by county, but commonly include:

- Dental coverage — routine cleanings, exams, and sometimes more extensive dental work.
- Vision coverage — routine eye exams and an allowance toward glasses or contacts.
- Hearing coverage — hearing exams and an allowance toward hearing aids.
- Over-the-counter (OTC) allowance — a periodic credit toward eligible health items like vitamins, pain relievers, or first-aid supplies.
- Part B premium giveback — some plans reduce or reimburse part of your monthly Part B premium as a plan benefit.

These extras are plan-specific, not guaranteed, and not available on every plan or in every ZIP code. TFL does not include most of these extras on its own. If added benefits like these are appealing, it's worth comparing exactly what a specific plan offers, alongside how it would handle TFL coordination (above) and your provider network, before making a change.

Before changing from Original Medicare to a Medicare Advantage plan, it's worth reviewing how the specific plan you're considering handles claims coordination with TFL and whether your preferred providers are in-network. This is exactly the kind of comparison an independent agent can walk through with you at no cost.

4. Filing a TRICARE For Life Claim (When It Doesn't Auto-Crossover)

If your claim doesn't cross over automatically — most commonly because you're enrolled in a Medicare Advantage plan — here's the general process:

1. Keep your Explanation of Benefits (EOB) from your Medicare Advantage plan and any receipts for what you paid out of pocket.
2. Get the current TRICARE claim form from [tricare.mil](https://www.tricare.mil) (or by calling the TFL claims contractor for your region).
3. Complete the form and mail it, along with a copy of your plan's EOB, to the TFL claims processor.
4. Allow the standard processing time before following up; keep copies of everything you submit.

Exact mailing addresses and contractor contact details are updated periodically by TRICARE. Confirm current instructions at [tricare.mil](https://www.tricare.mil) before mailing a claim.

5. VA Health Care and Medicare — Two Systems That Don't Talk to Each Other

This is the part that surprises the most people: VA health care and Medicare (Original or Advantage) do not coordinate benefits or share costs. You choose which system to use each time you seek care.

- Get care at a VA facility → your VA benefits apply.
- Get care from a non-VA provider who accepts Medicare → Medicare (or your Medicare Advantage plan) applies.
- The VA will not pay your Medicare Part B deductible or coinsurance, and Medicare will not pay for care delivered at a VA facility.
- Having Medicare does not reduce or replace your VA benefits, and having VA benefits does not exempt you from Medicare's enrollment deadlines or late-enrollment penalties.

6. Getting Reimbursed by the VA for Non-VA (Community) Care

The VA can pay for care you receive outside the VA system in two main situations — but the steps you take matter for whether you get reimbursed:

Planned (non-emergency) community care

This generally requires pre-authorization from VA before you receive care. Always confirm authorization with your VA Community Care office first; unauthorized non-emergency care is often not reimbursed.

Emergency care at a non-VA hospital

5. Notify VA within 72 hours of receiving emergency care or being admitted at a non-VA (community) hospital. Use the Emergency Care Reporting portal at EmergencyCareReporting.CommunityCare.va.gov, or call the VA Centralized Notification Center at 844-724-7842. If you're incapacitated, a family member or the hospital can notify VA on your behalf.
6. Once safe to do so, agree to transfer to a VA facility if VA requests it — declining a transfer can limit what VA will cover from that point forward.
7. To request reimbursement for out-of-pocket costs on emergency or unauthorized non-VA care, submit VA Form 10-320 to the VA Office of Community Care, along with your bills and supporting documentation.
8. If your care was pre-authorized through VA's Community Care Network, the claim is typically handled by the network's third-party administrator (currently Optum or TriWest) rather than filed by you directly — follow the instructions on your authorization.

The 72-hour notification window is strict, and missing it can jeopardize reimbursement. Write down any confirmation number you receive when you notify VA.

7. Key Takeaways

- TRICARE For Life pairs most seamlessly with Original Medicare — coordination is automatic and paperwork is minimal.
- You can keep TFL alongside a Medicare Advantage plan, but be ready to file paper claims and check network coverage.
- VA health care and Medicare are entirely separate systems — you decide where to seek care each time, and neither program reimburses the other.
- For non-VA emergency care, the 72-hour VA notification rule is the single most time-sensitive step in getting reimbursed.
- Every situation is a little different. Reviewing your specific coverage before AEP, before a VA authorization, or before any planned procedure can prevent costly surprises.

Questions about your specific situation?

Bryant Waldron | Independent Licensed Medicare Insurance Agent | Bryant Waldron Insurance

Serving Richmond, Sugar Land & the Greater Houston / Fort Bend County area, Texas

713-882-7196 • bryant@bwaldron-insurance.com • NPN 8221947

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